OPTM 8513/8523 Clinical Ocular Therapy Oral Examination

Weighting: 35% of your final grade. Hurdle Assessment - A pass mark in the oral examination is required to pass the course

Date: Monday 23rd November 2026 - Thursday 26th November 2026. See oral examination schedule for allocated date and time

Format: In-person, oral examination with 2 examiners

Location: Lederer Meeting Room 3.001

Length: Approximately 20 minutes

Assessment description:

Candidates will be assigned a date for their oral examination by the School. There will be two therapeutically qualified examiners. The length of the oral examination is approximately 20 minutes, but this may vary.

The oral examination will be based on cases amendable to optometric management, written case reports, legislative issues and other topics of therapeutic management. The examination assesses the candidate's ability to justify and explain safe clinical decisions including the use of therapeutic agents.

Assessment outcomes:

To achieve a pass mark in the oral examination, candidates must demonstrate safety and competency in ocular therapeutics in an Australian and/or New Zealand setting.

Assessment tips (but not limited to):

- The oral examination may include ocular pathology slides/images.
- Be prepared for quick changes in topic – there is a lot to cover in 20 minutes
- Expect to be presented with alternative patient or history scenarios when discussing treatments including special populations
- Re-read your case report and reflection task feedback before the exam. These may form part of the questions and may be used to open the discussion
- Be able to interpret clinical information presented to them including images of the ocular structures, visual pathway, or diagnostic printouts such as visual fields or OCT
- Be able to consider and exclude relevant differential diagnoses in disease
- Be familiar with the Optometry Board of Australia Guidelines for Use of Scheduled Medicines
- Understand the mechanism of action of drug classes
- Be able to discuss contraindications, precautions, adverse effects and any potential drug interactions of medications
- Understand the ocular effects of systemic drugs and systemic effects of ocular drugs
- Understand the appropriate dosage, frequency, follow up period as well as approach if treatment response is not expected.
- Be able to apply grading systems and use appropriate nomenclature where appropriate
- Be familiar with key clinical trials and guidelines and be able to explain how their findings apply to clinical management.
- Understand the endpoints for treatment and referral indications

RED FLAG PRINCIPLE

Candidates are to be aware of the 'red flag' principle. A fail grade on the oral examination may be given regardless of satisfactory performance in other aspects if a proposed management plan is considered sight or life threatening. It is thus crucial for candidates to be aware of the absolute and relative contraindications of the use of medicines, management of ocular emergencies and situations which require immediate referral.

Marking rubric: Maximum mark = 20*

Category	Excellent (5)	Satisfactory (3–4)	Borderline (1–2)	Unsatisfactory (0)
Clinical interpretation and work-up (25%)	Accurately interprets key findings and effectively identifies, selects and justifies further history, examination and investigations, supported by relevant clinical knowledge and evidence-based practice.	Interprets majority of key findings and selects an appropriate diagnostic work-up with generally sound justification. Demonstrates satisfactory clinical reasoning, with only minor omissions or occasional prompting.	Interpretation is incomplete or contains significant errors. Missing further investigations with limited justification. Requires substantial prompting.	Unable to meaningfully interpret the key findings or identify essential further assessment, despite prompting.
Diagnosis and clinical reasoning (25%)	Establishes a well-reasoned and evidence-based diagnosis, with clear justification for excluding relevant differentials. Accurately applies appropriate classification, grading and diagnostic terminology.	Identifies an appropriate working diagnosis and relevant differential diagnoses, with generally sound justification. Minor prompting may be required to develop or refine clinical reasoning.	Offers a plausible diagnosis with weak or inconsistent justification. Differentials are incomplete, limited or poorly prioritised. Requires substantial prompting.	Unable to establish a defensible diagnosis or relevant differentials, despite prompting.
Management and therapeutic prescribing (25%)	Formulates a safe and evidence-based plan. Provides clear justification for treatment choices using relevant evidence or guidelines and understands contraindications, precautions, interactions and adverse effects. Addresses relevant legislation requirements, and identifies referral indications, urgency and co-management where required.	Proposes a safe, appropriate plan supported by generally correct prescribing details and sound rationale. Demonstrates appropriate awareness of evidence, legislation, scope of practice and referral indications, with minor omissions or occasional additional prompting.	Plan is partially appropriate however has significant omissions or prescribing errors are evident. Limited application of evidence-based practice, incomplete understanding of legislation, scope of practice or referral indications. Requires substantial prompting to develop a safe management plan.	Unable to formulate an appropriate plan, or persists with fundamentally unsafe management, prescribing or referral decisions despite prompting.
Follow up, prognosis and ongoing care (25%)	Specifies appropriate review timing, monitoring and treatment endpoints. Clearly explains the expected clinical course, risk of recurrence and complications. Justifies management changes in response to treatment failure, adverse effects or disease progression including referral or escalation of care where appropriate.	Provides an appropriate follow-up plan and a realistic prognosis. Identifies the main monitoring needs and circumstances requiring escalation of care, with minor omissions or occasional additional prompting.	Provides an incomplete or vague follow-up plan. Limited understanding of prognosis, monitoring strategies and indications for escalation. Requires substantial prompting to address nonresponse to treatment or disease progression.	Provides an inappropriate, or no follow-up plan, or fails to recognise essential monitoring or treatment escalation needs, despite prompting.

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